



Electroconvulsive Therapy (ECT) (Acute) Policy and Procedure

- ALERT**
- ECT will be prescribed in accordance with the:
 - * [Mental Health Act 2014](#)
 - * [The E.C.T. Guide: The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2006](#)
 - * [Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy 2015](#)
 - * [Royal Australian and New Zealand College of Psychiatrists \(RANZCP\) professional practice guidelines for the administration of electroconvulsive therapy 2019](#)
 - * [Chief Psychiatrist's Standards for Clinical Care](#)

Preamble

Consumer Participation

The Rockingham Peel Group (RKPG) recognises and fully supports the need for the patient and/or their primary carer to¹:

- Know and exercise their healthcare rights.
- Be engaged in their healthcare.
- Have access to information about treatment options.
- Participate in treatment decisions.
- Have access to information about agreed treatment plans.

Scope of Practice

It is the responsibility of each health care provider to ensure all patient care and therapeutic interventions they provide are within their individual scope of practice. Refer to the [RKPG Guidelines to Writing for Clinical Policy](#) for further information about Scope of Practice.

Definition

Electroconvulsive therapy (ECT) uses a small electric current to produce a generalised cerebral seizure under general anaesthesia (GA). ECT is used mainly to treat severe depression, but is also indicated for patients with other conditions, including bipolar disorder, schizophrenia, schizoaffective disorder, catatonia, and neuroleptic malignant syndrome.

Aim

The purpose of this document is to facilitate a smooth and safe operational process for ECT treatments ensuring all patients requiring ECT treatment will be managed in accordance with the [Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy 2015](#) as set out by the Chief Psychiatrist of Western Australia and [Royal Australian and New Zealand College of Psychiatrists \(RANZCP\) professional practice guidelines for the administration of electroconvulsive therapy 2019](#).

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Standards for the administration of ECT

- The treating Consultant Psychiatrist takes the ultimate clinical responsibility for the decision to treat a patient with ECT.
 - * This decision will be made in conjunction with the team responsible for the care of the patient, and the Anaesthetist.
 - * The Anaesthetist will be responsible for determining the physical fitness of a patient to undergo general anaesthesia.
- ECT shall be given in a specified place equipped with anaesthetic and resuscitation equipment. The Endoscopy Suite is to be utilised for this purpose at Rockingham General Hospital (RGH).
- RKPG ECT service is available on a Monday, Wednesday and Friday commencing at 08:00hrs.
- ECT may be conducted on a public holiday falling on Monday, Wednesday, Friday and this will normally be done in the Main Theatres in RGH or as advised by the Theatre Nurse Unit Manager (NUM).
- In all cases there will be two senior medical staff present, one of whom shall be a Consultant Psychiatrist credentialed in the administration of ECT and the other an Anaesthetist.
- An anaesthetic technician proficient in resuscitation procedures will be present. Following each ECT the Anaesthetist will delegate responsibility to Post Anaesthetic Care Unit (PACU) staff, where the patient will be managed as per the criteria for discharge following an anaesthetic.
- All practitioners administering ECT will be conversant with the indications and side effects of the treatment, and the anaesthetic emergencies and consequent resuscitation procedures, before they assume such responsibility.
- The Consultant Psychiatrist and Psychiatric Registrar/medical officer (MO) involved in administering ECT must have mastered the knowledge levels or competencies identified by the Chief Psychiatrist ([The E.C.T. Guide: The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2006](#)).
- ECT Patient referrals sent from external services will be sent to the Peel and Rockingham Kwinana Mental Health Medical Co-director who will then liaise with the admitting Consultant Psychiatrist responsible for accepting the out of area referral. The admitting Consultant will assess the suitability of the patient for the purpose of ECT and discuss with the Medical Co-Director. Once agreed to proceed with ECT, the admitting Psychiatrist will assume responsibility of the patient's care including prescribing and making decisions about the ongoing appropriateness of ECT treatment.
- The ECT Machine (Thymatron® System IV) will be registered on the Australian Register of Therapeutic Devices by Biomedical Engineering.
- The ECT machine will be reviewed annually against current research evidence and updated if it no longer meets best practice requirements.

Escalation

- If the patient due to have ECT medically deteriorates the Shift Coordinator must escalate to the Treating Team Consultant or out of hours On-call Consultant Psychiatrist for a decision as to whether it is still appropriate and safe for the patient

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to have ECT. This decision must be made before escorting the patient for ECT and must be documented in the medical record.

Informing/education of patients and carers

- As soon as a decision is made for a patient to receive ECT the ECT Coordinator must be informed to allow time for appropriate education to the patient and family.
- Patients and their carers/personal support persons must be provided with appropriate information about the administration of ECT and have the opportunity to discuss their concerns and ask questions. (ECT fact sheets from the Black Dog Institute, RANZCP, or Beyond Blue are available on the ward along with an educational DVD).
- The discussion, including advantages and disadvantages must be clearly documented in the medical record. Treating Psychiatrists should record in the patient's medical record matters which have been discussed including potential adverse effects and risks. Questions asked by the patient or family and the answers provided should be documented.
- Patient information will be placed in Mimidi Park waiting area and distributed to patients as part of their psychoeducation around ECT
- In circumstances where English is not the first language for the patient or carer, all aspects of communication regarding ECT, including issues of consent, must occur with approved interpreters present during patient review. This process must also be applied to include people who are deaf, hard of hearing, Aboriginal Australians and people from culturally and linguistically diverse backgrounds. This is to ensure that the patient and carers are fully informed in line with legislation and policy ([WA Health Consent to Treatment Policy](#)).
- The rights and responsibilities of consumers being treated with ECT will be documented, applied and promoted throughout all phases of the treatment.

Specific information for consent

- Prior to attending for ECT the patient's Mental Health ACT status must be clearly documented on the Pre-operative handover checklist RGMR94E.
- This must include involuntary or voluntary, and, if involuntary, whether or not the patient has been approved for emergency ECT.
- **Emergency ECT**
 - * Emergency treatment will be provided in accordance with the Mental Health Act 2014 Division 1 s199 and must be approved by the Chief Psychiatrist or their delegate (For RKPG this is the Mental Health Medical Co-Director).
 - * **While applying for emergency ECT through the RKPG Mental Health Medical Co-Director, the prescribing Psychiatrist must also apply for Mental Health Tribunal approval of ECT.**
 - * **Every emergency ECT** session prior to Mental Health Tribunal approval of ECT must have individual documented approval by the RKPG Mental Health Co-Director both documented and filed in the medical record prior to any ECT occurring.
- **Involuntary patients:**
 - * Must be informed of their rights and the Treating Team (prescribing Psychiatrist) must make an application to the Mental Health Tribunal (MHT) who approve or revoke the recommendation of the Treating Team. If approved

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the authorisation from the Mental Health Tribunal must be documented on the ECT consent form (RGMR20Q) (MHA 2014 s198 (2) (a); s199) and filed in the patient's medical record.

- * An involuntary patient (or someone subject to the [Mentally Impaired Accused Act \(MIA\) 1996](#)) can **only** have ECT if the Mental Health Tribunal provides approval. Informed consent is not required but the MHT approval is required.
- * The ECT Treating Team (prescribing Psychiatrist) is required to submit an application to the Mental Health Tribunal in writing. Treatment cannot commence until approval has been obtained.
- **Voluntary patients:**
 - * The patient should be informed that in consenting to a course of ECT they are able to withdraw consent at any time during treatment. Patients and carers should be made aware that consent is for a course of treatments and the number of treatments is to be specified.
 - * In addition to the Consent for ECT therapy (RGMR20Q), voluntary patients will also require Consent to anaesthesia (RGMR20E)
- **Note:** The ECT Consent form (RGMR20Q) and the ECT prescription form (RGMR94G) should be renewed every 3 months or 12 treatments, whichever occurs first. A new consent form is required if the placement of electrodes is changed.

Pre ECT preparation and assessment

- The decision to commence ECT treatment is made by the Admitting Inpatient Consultant Psychiatrist responsible for the care of the patient. The Consultant must document the reason for ECT in the patient's medical record including the ICD diagnostic code.
- The ECT Coordinator will identify the need for additional service support i.e. interpreter services and coordinate the appropriate services.
- Patients prescribed ECT will be referred to the Anaesthetist for pre-anaesthetic assessment. The treating team Registrar or MO is responsible for arranging anaesthetic review and should ensure all pathological investigations are available before anaesthetic consultation
- The Anaesthetist is responsible for determining the physical fitness of a patient to undergo general anaesthesia. All documented information including the history, physical examination, results of blood tests, x-rays and electrocardiogram (ECG) will be readily available, in order that they may have adequate information on which to base such a decision.
- In cases where doubt exists about a patient's suitability for ECT, such as in recent myocardial infarction, head injury, chest infection and complicated chronic obstructive pulmonary disease, the opinion of a specialist Physician will be sought.
- All patients must have a medical assessment clearly documented in the medical record prior to the commencement of ECT.
- The baseline assessment will be conducted no more than 7 days prior to the commencement of ECT
- All consumers must have a documented treatment, support and discharge plan (TSDP) in the health record. Where possible the patient will be involved in this process and given the opportunity to read and sign the plan.

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- The Consent for ECT Therapy (RGMR20Q) must be fully completed and specify both the number of treatments prescribed and the specific electrode placements.

Medical assessment to include but not limited to

- Complete full medical history.
- A standard physical examination carried out prior commencement of ECT. These investigations should be current within 7 days prior to commencing a course of ECT.
- ECG.
- Blood biochemistry screening: requested blood biochemistry results including quantitative blood pregnancy test (or a beta HCG test) for females of child bearing age.
- ± Chest x-ray as indicated by treating team Registrar/MO or Anaesthetist.
- +/- Electroencephalogram (EEG).
- An evaluation of dentition, for the presence of dentures and/or dental problems that could affect the use of the bite block. Any temporal-mandibular joint problems are noted in the patient's medical record.
- Any other relevant investigation i.e. CT/MRI will be performed based on assessment by the treating team Registrar/MO.
- If the patient is a smoker, nicotine replacement therapy will be offered.

Rating Scales to be completed during the course of ECT

- In summary this includes the Montgomery Asberg Depression Rating Scale (MADRS), the Clinical Global Impression Scale (CGI), the Montreal Cognitive Assessment (MoCA) and the Post Electroconvulsive Therapy orientation assessment.
- The MADRS (RGMR94.1) is completed and scored pre and post ECT course by the treating team Registrar / MO for patients with depression.
- The CGI scale (RGMR94.2) will be completed by the Registrar/MO pre, mid and post the ECT course. This assessment gives a view of the patient's global functioning prior to and after initiating ECT treatment.
- All patients receiving ECT treatment will have the MoCA prior to commencement of the course of treatment and after completion of the course of treatment to evaluate cognitive changes. This assessment is to be completed by the treating team Registrar/MO who are versed that in its use.
- The orientation of the patient post ECT will be assessed using the Post ECT orientation assessment (RGMR94.H) 30 minutes after receiving ECT. This assessment will be completed by the nursing staff involved in the recovery of the patient post treatment (this can be either PACU staff or the ECT Coordinator).

Medications during ECT

- It is the responsibility of the treating Psychiatrist to review the patient's medications and recommend what medications need to be adjusted or ceased before commencing ECT.
- Identify, in the patient integrated notes, which medications should be withheld during each ECT treatment, which medications should be given on the morning of ECT, and which medications should be continued post ECT. Amendments must also be made

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to the patient's medication chart. The ward Pharmacist will be involved in this process.

Benzodiazepines

- Benzodiazepine tranquilisers and hypnotics are not recommended during ECT given their anticonvulsant nature and hence it is advisable to cease them.
- If ceasing them is not possible, it is advisable to use shorter acting benzodiazepines (e.g. lorazepam) and to withhold them the night before until completion of the ECT session the next morning.

Anticonvulsants (e.g. Carbamazepine, Sodium Valproate, Lamotrigine, Pregabalin)

- Anticonvulsants increase the seizure threshold and therefore reduction or cessation of these medications needs to be considered where possible, particularly when used as a mood stabiliser. However, patients with epilepsy may need to continue the treatment with anticonvulsant during the course of ECT and temporary reduction in the dose needs to be considered if it safe to do so.
- It is recommended that the treating team seeks input from a neurologist prior to altering anticonvulsant medications where it is prescribed for a neurological condition.

Lithium

- There is no absolute contraindication to concomitant use of Lithium during a course of ECT however, combining the two can sometimes cause confusion and elderly patients are at increased risk of developing delirium.
- The general principle is to suspend Lithium during the course of ECT. If this is not possible Lithium can be continued at lower therapeutic dose/or withhold the dose the day before ECT

Antihypertensives

- Any hypertensive medication should be continued and administered to the patient 1-2 hours prior to ECT with sips of water. However consider administration of diuretics post ECT treatment to minimize amount of urine in the bladder and prevent urinary incontinence as a result of the seizure.

Bronchodilators

- Medications for the treatment of asthma to be given prior to ECT treatment to prevent bronchoconstriction.

Anti-reflux meds

- Anti-reflux medications should be given 2 hours prior to ECT treatment to prevent reflux and possible aspiration.

Diabetic medication

- Medications for diabetes mellitus will be managed as per hospital protocol for patients coming for minor procedure ([RKPG Peri-operative Diabetes Management - Adult \[Acute\] Guideline](#)).

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- Oral hypoglycaemics can usually be administered once the patient has recovered from the effects of anaesthesia just prior to resuming oral intake. One may need to consult a Physician or Endocrinologist to seek advice on the insulin dose however this is generally administered post recovery from the ECT session before breakfast.

Antiplatelet and anticoagulant medications

The management of antiplatelet and anticoagulant medications will have to be discussed at the time of pre assessment as would be any other medications of clinical relevance.

All other regular medications

- All regular medications need to be continued unless specified in the pre-anaesthetic assessment, precluded by the procedure itself or otherwise documented by the treating Psychiatrist. All regular medications are to be administered when the patient returns from the ward from ECT as charted.

Booking/cancelling and scheduling ECT

- For patients in the Mental Health In patient Unit requiring ECT treatment, the Consultant Psychiatrist/Psychiatric Registrar/MO will notify the NUM and ECT Coordinator at least 24 hours prior to ECT of the following details before ECT commences:
 - * Name.
 - * Age.
 - * Legal status.
 - * If it is emergency ECT
 - * Proposed frequency of treatments.
- The ECT Coordinator will follow up and check all relevant documentation has been completed including the Electroconvulsive Therapy Treatment form (RGMR94G) before making any bookings with theatre.
- Theatre bookings for ECT should be made 24hrs prior to the procedure. In the event of any changes the ECT Coordinator/NUM must liaise with the Theatre NUM as soon as possible for any changes to the scheduled list.
- Theatre can only be booked
 - * Once the voluntary patient has signed their consent forms.
 - * Or approval has been received for involuntary patients by the Mental Health Tribunal.
 - * Or Chief Psychiatrist/Delegate RKPGE Mental Health Medical Co-Director for emergency ECT.
- The completed ECT Treatment order form (RGMR94G) is given to the ECT Coordinator or in his absence the ward clerk to be faxed immediately to the Pre Admission Clinic (PAC).
- The ECT Coordinator completes a Theatre booking form located in W drive then it is emailed to RKPADM.RKPADM@health.wa.gov.au for booking processing. The booking form can be accessed by all medical staff.
- The ECT Coordinator completes the Theatre booking forms every Friday for the following weeks scheduled ECT.
- Theatre must be advised of any future changes/cancellations to these bookings by either the NUM or ECT Coordinator as soon as possible.

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Patient preparation/management in Mental Health Inpatient Unit pre ECT

Twenty four hours prior to treatment

The ECT Coordinator in consultation with the NUM will assess the care level and coordinate and document the escorting requirements of the patient based on current clinical risk.

Evening shift staff prior to ECT, allocated nurse responsibility

- Ensure the ECT procedure has been explained to the patient by the Treating team Consultant Psychiatrist/Registrar/MO and written information on ECT has been provided and document in patients' medical records.
- Ensure that routine blood tests and ECG/scan results are available and have been reviewed by the medical staff.
- Prior to every ECT session the legal status of the patient as defined by the Mental Health Act 2014 must be checked to ensure there is valid consent in place.
- For Involuntary patients check that the Mental Health Tribunal approval is present in the patient notes and documented on the Consent to Electroconvulsive Therapy (RGMR20Q).
- For emergency patients check that the Chief Psychiatrist delegate approval from Peel and Rockingham Kwinana Mental Health Medical Co-director is documented in the medical record with the approval email in the medical record. This is required for **every** single emergency treatment.
- For voluntary patients Check that the Consent for Electroconvulsive Therapy (RMGR20Q), Patient Consent to Anaesthesia (RGMR20E) and the Anaesthesia Record (RGMR94A) are completed. Ensure old medical records are on the ward.
- Ensure sufficient patient identification labels are available with the patient's medical records.
- Ensure the patient is wearing two correct patient identification bands (no addressograph). If the patient has a medication allergy red patient identification bands are used.
- If the allocated nurse identifies any consents have not been signed this must be escalated to the treating team. Action to obtain consent must occur prior to the patient attending ECT.
- If the allocated nurse identifies any missing approvals from the Tribunal for Involuntary ECT or OCP Delegate Peel and Rockingham Kwinana Mental Health Medical Co-director for emergency ECT this **must** be escalated as ECT would need to be cancelled.
- The allocated nurse is to ensure the patient is fully aware of the fasting period, which is 6 hours prior to ECT procedure; patients may have clear fluids up to 2 hours before procedure. Clear fluids include water, clear apple juice and cordials.
- The allocated nurse is to encourage the patient to wash hair with shampoo the night before procedure. Hair should be free of any conditioners, hair products or hairpins to prevent an increase or excessive decrease in electrical resistance between the treatment electrodes and scalp. Dry shampoo may be utilised if hair is excessively oily or unclean.
- The patient should be reminded not to wear makeup or creams on their face, no talcum powder on their body and staff to remove nail varnish if present.
- Ask the patient if they have any body piercings. These will have to be taped or removed prior to ECT.

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Night staff, allocated nurse responsibility

- Ensure the patient fasts for 6 hours before each scheduled treatment.
- Jewellery is to be removed and stored as per the [RKPG Patients' Property – Management in Mimidi \(Mental Health\) Guideline](#). The jewellery is to be offered back to the patient after ECT treatment. The patient may retain a wedding band which is to be covered with tape prior to treatment.
- If the patient smokes they will be advised of the need to refrain from smoking from midnight prior to the treatment. Nicotine replacement therapy is to be offered to the patient and recorded in the patient notes.
- Any hypertensive medication should be dispensed to the patient 1-2 hours prior to ECT and can be administered with sips of water.
- Place all documentation for ECT in the front of the patient's medical record in the plastic sleeve provided. Documentation includes:
 - * Notification from Mental Health Tribunal for involuntary patients.
 - * Email from OCP delegate Peel and Rockingham Kwinana Mental Health Mental Health Medical Co-Director for emergency ECT.
 - * Consent for Electroconvulsive Therapy (RMGR20Q).
 - * Patient Consent to Anaesthesia (RGMR20E).
 - * Anaesthesia Record (RGMR94A).
 - * Pre-operative Handover Checklist (RGMR94E).
 - * Surgical Safety Checklist (RGMR95.C).
- Cognitive measures utilised are:
 - * Montgomery-Asberg Depression Rating Scale (MADRS) (RGMR94.1).
 - * Montreal Cognitive Assessment (MoCA).
 - * Clinical Global Impression Scale (CGI) (RGMR94.2).
 - * Post Electroconvulsive Therapy Orientation Assessment (RGMR94.H)
- Ensure the patient's medications charts are placed together with the patient's notes and ensure there are sufficient patient identification labels readily available in the patient medical record.

Day staff allocated nurse responsibility

- Ensure patient has fasted from solid food inclusive of milk, tea, coffee, pulp and juices, for 6 hours pre-ECT, patients may have clear fluids up to 2 hours before procedure. Clear fluids include water, clear apple juice and cordials.
- Nursing staff to check the journey board and patient's health record to ascertain MHA 2014 status of patient before patient has ECT treatment. Proceed to Pre-operative checklist RGMR94E and under mental health status, tick the relevant status as well as if whether emergency ECT is being employed.
- As a safety mechanism the ECT Coordinator will follow this process to check and confirm patients MHA 2014 status in the morning prior to each treatment session. If in doubt, please check with the Shift Coordinator or treating Psychiatrist. If the ECT Coordinator is on leave this falls to the acting ECT Coordinator.
- For Involuntary patients, ensure a copy of the Mental Health Tribunal letter approving provision of ECT to an involuntary patient is present in the medical record prior to ECT treatment.

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- For emergency ECT ensure correspondence and authorization from the Chief Psychiatrist/Delegate Peel and Rockingham Kwinana Mental Health Medical Co-director is documented in the medical record prior to ECT treatment.
- For voluntary patients verbal consent for ECT treatment should be obtained from patient and documented in the medical record prior escorting patient to Endoscopy suite.
- Ensure the Pre-operative Handover Checklist (RGMR94E) has been completed by night staff in the morning before the procedure.
- Ensure the patient's baseline observations are recorded, including blood sugar levels if diabetic. Any abnormalities in these observations and any other overt/identified medical problems are to be communicated to the ECT Coordinator before leaving the unit.
- Ensure one patient ID band to the wrist, one band to the ankle is applied on the patient for identification purposes as per RKPG Patient Identification (Acute) Policy.
- The patient identification process for the patient name band requires two staff verifying the patient's details along with the health record.
- The patient is encouraged to void urine and empty bowels prior to treatment. Assess the patient's continence status and manage accordingly.
- Patients are to be dressed in loose, warm and comfortable clothing.
- Allow the patient to retain their contact lenses/glasses, hearing aids, and dentures until immediately before the ECT procedure. These items should not be removed until the patient's privacy and dignity can be assured. These items must be kept safe during the procedure and can remain with the patient to aid communication. Label these items where possible with a patient identification label.
- If the patient has dentures, leave them in situ as they will be taken out just before procedure.
- The ECT Coordinator will contact the Mental Health In patient Unit to arrange for the patient to be escorted to the Endoscopy suite.

Escort nurse

- The allocated nurse escorting the patient to the Endoscopy suite will provide a supportive environment for the patient, ensuring proper preparation of the individual patient, and a clear handover to the ECT Coordinator.
- The escort nurse can walk the patient to the Endoscopy suite or use a wheelchair if required. The electric wheelchair will be used to meet the mobility needs of bariatric patients and therefore maintain safety of staff.
- The escort nurse hands over the patient to theatre staff utilising the Pre-operative Handover Checklist (RGMR94E) and confirms consent with patient. This form should be signed by the nurse giving handover and the nurse receiving handover.
- If a patient has taken any sedative prior to ECT, this should be reported to the ECT Consultant Psychiatrist and an entry made in the patient's medical file
- Remain with the patient in the ECT waiting area until the patient transfers to the ECT suite.

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Prescription of ECT

- The treating inpatient admitting Psychiatrist at Mimidi Park will determine the electrode positioning for the treatment.
- The ongoing prescription of specific energy doses to be delivered will be the responsibility of the treating team Consultant Psychiatrist.

Equipment for ECT treatment

- The Thymatron® System IV machine is used to administer ECT treatment at RGH. The machine is registered with the Therapeutic Goods Administration Medical Devices.
- The machine will enable EEG monitoring of seizures.

Thymatron® testing

- The Thymatron® System IV automatically tests the integrity of all circuits. When the unit is powered on, a flashing nonsense symbol appears for several seconds in the 8-character L.E.D., followed by the flashing message “SELFTEST” for a few seconds, then a self-test confirmation report is printed and the words “NO BASE” appear.
- Prior to commencement of the ECT list, the treating Consultant and ECT Coordinator will ensure the machine is functioning. Testing will be performed weekly on Fridays and the Safety Monitor Circuit test is performed as follows:
 - * Power on the Thymatron® System IV and connect the ECT stimulus cable.
 - * Rotate the ‘PERCENT ENERGY’ dial to any setting.
 - * Connect the ECT stimulus cable banana plugs to a 200 ohm, 10 watt load resistor (or insert these banana plugs into the designated jacks of the ECTOBRAIN™ II testing device).
 - * First press and hold down the rear panel ‘ALARM TEST’ button. Next, press and hold the ‘TREAT’ button, and then release the ‘ALARM TEST’ button.
 - * Continue pressing the ‘TREAT’ button while the Thymatron® System IV goes through the full cycle of stimulus warning signal and stimulus indicator tones, then release the ‘TREAT’ button.
 - * At the end of the stimulus indicator tones the ‘SAFETY MONITOR ACTIVATED’ dot L.E.D. will go on and a high-pitched, continuous signal tone will sound until the unit is powered off. This shows the alarm signal is operating correctly. If the indicator light and alarm signal tone do not occur, do not use the unit to treat patients until it has been examined and cleared by authorised biomedical personnel.
- Any faults with the machine will be reported to the hospital Equipment Clinical Nurse (CN).

ECT in Endoscopy Suite/theatre

- All resuscitation equipment should always be available and checked daily by theatre staff as per the RKPG Resuscitation Trolley (Adult) – Equipment and Checking (Minimum Standard) (Acute) Policy.
- The Mental Health Inpatient NUM or ECT Coordinator will notify the Operating Theatre Coordinator or NUM of any potential clinical issues, which could affect the

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smooth running of the ECT sessions.

- Theatre staff together with ECT Coordinator will verify the consent forms including the name of the procedure and name of the patient from the accompanying documentation. Patient identification will be checked by using the three core identifiers, family and given name, UMRN and date of birth (DOB). The patient will also be asked to confirm the name of procedure.
- Verification (Team Time-out) process for the patient, site and procedure identification will apply to ECT patients prior to the commencement of anaesthetic or medication that could affect the patient's cognitive function using Surgical Safety Checklist (RGMR95C).
- The treating Consultant, Anaesthetist and Psychiatry Registrar/MO will document and sign the ECT Patient Register and ECT Record after each ECT session.
- Mental health staff working in the delivery of ECT should follow the RGH Clinical Pathway for Novel Coronavirus COVID-19 protocol with regards to fitness to work in respect to COVID pandemic, COVID screening and PPE use. All ECT patients should be screened for COVID-19 risk as per this protocol.

Medical staff administering ECT

- Practice of Anaesthesia should follow the guidelines of the Australia New Zealand College of Anaesthetists. In all cases at least one MO skilled in administration of anaesthesia will be present during the procedure.
 - ECT will be administered by a Psychiatrist or a Psychiatric Registrar under the supervision of a Psychiatrist. Psychiatry trainees shall satisfy the RANZCP training requirements for administering ECT. A record of participation in ECT administration by the Rostered ECT Consultant and Registrar will be captured on the Rockingham General Hospital ECT Treatment Record.
 - All Registrars and MOs working at the Mimidi Park inpatient unit will be rostered on the service medical roster to deliver ECT treatment under the supervision of the Rostered ECT Consultant Psychiatrist.
 - Medical staff administering ECT should meet the SMHS Credentialing and defining the scope of clinical practice for medical, nursing and midwifery practitioners... requirements as stipulated in the [The E.C.T. Guide: The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2006](#).
 - It is the medical staff responsibility to press the button on the ECT machine.
 - The key responsibilities of this role include:
 - * Ensuring that the chosen electrode sites are thoroughly cleansed with alcohol soaked gauze squares to remove oil, make up, gel from previous treatments, hair sprays, dead skin cells etc. Attention is required to ensure skin is not abraded.
 - * Apply a conductive ECT gel onto the electrode surfaces.
 - * Firmly press the electrodes against the skull. This is essential to minimise impedance which should be measured and should be less than 3 kilohms.
- Note:** it is not necessary to shave the hair. In the event that a parietal site is used for unilateral ECT, hair can be parted and cleaned as described.

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Electrode placement

Bi-temporal (bilateral) electrode placement

- Electrodes are placed over both temples. The midpoint of each electrode is placed 3 centimetre (cm) above the midpoint of an imaginary line drawn between the external canthus of the eye and the tragus of the ear on the left and right temples.
- This provides the most effective treatment response - however has the most side effects including disorientation.

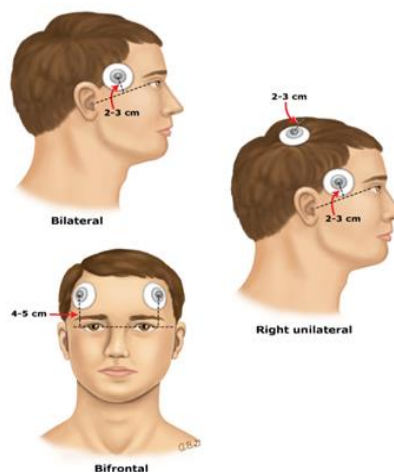
Bi-frontal electrode placement

- This method is the most often used. It is moderately effective with moderate side effects.
- The anterior EEG recording electrodes should be moved medially to approximately one cm either side of the midline to allow room for placement of the treating electrodes. Concave metal electrodes or adherent disposable electrodes must be used. The midpoint of each treating electrode is placed five cm above the outer canthus of each eye, in a parasagittal plane.
- It is recommended that the correct site be identified by measuring the distance above the outer canthus rather than estimating the distance.
- Care must be taken to avoid any contact between the treating electrodes and the EEG recording electrodes, and that there is no excess conductive gel creating a short circuit of the current across the forehead. Note that skin burns may occur if the electrodes are placed too closely together.

Right unilateral electrode placement

- This method is the least often used. It is the least effective but has the least associated cognitive side effects. The temporal electrode is placed over the right temporal fossa, with the centre of the electrode 2.5 cm above the midpoint of a line drawn between the tragus and the outer canthus of the eye. The centre of the second electrode is placed slightly (one cm) to the right of the vertex, which is at the intersection of the line drawn between the nasion and the inion (occipital process), and the line drawn between the tragus of each ear.
- For left unilateral ECT, the same positions are used, but on the left side of the head.

Electrode positions



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Steps to reducing impedance

- Prior to each ECT, inspection of each electrical connection in the treatment lead circuit should be done as they may be loose or corroded.
- Check the electrodes have not slipped or twisted.
- Check that there is adequate gel under the electrodes.
- Reposition electrodes to minimise the amount of hair underneath.
- Increase pressure on the treatment electrodes by firmly pressing electrode handles on top of the Thymapads®.
- If Thymapads® are being used, switch to Electrode Holder with metal electrodes attached as they have intrinsically higher impedance.
- If impedance reading remains at 3,000 ohm after the above procedures have been carried out, treatment may be given, but this problem should be investigated further by contacting the biomedical engineering to check the integrity of the treatment circuit.

Steps to be followed during ECT treatment administration

- It is imperative to establish safety standards to ensure patient safety by preventing delivery of treatment during a premature conscious state. The following process will be followed at all times when administering ECT treatment:
 - * Anaesthetic technician will apply monitoring equipment, while the Anaesthetist inserts an intravenous (IV) cannula and once inserted then Anaesthetist will advise Psychiatrist/Psychiatric Registrar that the cannula has been inserted.
 - * Psychiatrist/Psychiatric Registrar will apply EEG electrodes and Thymapads® on the patient while ECT Coordinator applies Thymatron® ECG electrodes.
 - * The Registrar will obtain a baseline EEG printout by pressing the black start button to the top left of the printout on the Thymatron® IV. Press the black start button again to stop the printout.
 - * When the impedance readings are satisfactory the Registrar will inform the Anaesthetist, and confirm it is time to commence anaesthetic for ECT.
 - * The Anaesthetist will administer appropriate anaesthetic medication and when the patient is completely anaesthetised a muscle relaxant will be administered.
 - * The Anaesthetist and ECT Coordinator confirm the anaesthetic and muscle relaxants are working (by waiting until fasciculation is over). The Anaesthetist confirms that it is safe to proceed with ECT.
 - * Once the patient mouth guard is in place the Registrar/MO will confirm readiness to proceed by stating "Ready to treat" (the mouth guard is inserted with the narrow end projecting forward through the patient's lips).
 - * The Consultant Psychiatrist or supervised Psychiatric Registrar/MO will then proceed and check impedance by pressing and holding down the Impedance button until a number value shows on the display. The Psychiatrist will confirm "OK to treat now?" When an affirmative reply is given by Anaesthetist and ECT Coordinator, the treating Psychiatrist will call out "Treating" and press and hold the yellow treatment button until treatment sounds have finished.
- **Note:** A laminated copy of the above steps is available on the ECT trolley and always will be displayed during ECT treatment for reference.

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Stimulus dose strategies

The 'dose titration method' involves stimulating the patient with a gradually increasing electrical dose, starting at a low dose to determine threshold for an induced seizure. The dose increases are delivered until an adequate seizure is obtained. Adequacy is determined via EEG morphology from the EEG readout delivered by the ECT machine. This method allows the seizure threshold to be estimated at the first treatment. Stimulus dosing is then carried out with respect to that estimate at successive treatments.

Seizure monitoring

Effectiveness of a seizure will be monitored by measuring:

- Duration of seizure.
- Higher amplitude spike and wave activity during the seizure.
- Sharp post - ictal suppression.

Missed seizures

Missed seizures are associated with an increased likelihood of post stimulus asystole, leading some practitioners to add an anticholinergic premedication. ²

Prolonged seizures

- Most ECT seizures are self-limited, lasting less than 2 minutes. However persistent seizure activity (i.e. prolonged seizure) may occur.
- A prolonged seizure can be defined as a seizure activity lasting longer than 3 minutes. EEG monitoring is especially useful in detecting prolonged seizures because most are undetectable by motor observation alone.
- Prolonged seizures are particularly likely to occur in one of the following circumstances:
 - * At the first treatment.
 - * During benzodiazepine withdrawal.
 - * In patients whom pro-convulsant medications (e.g. caffeine) and lithium are used concurrently.
- Prolonged seizures should be terminated pharmacologically after 3 minutes of sustained seizure activity.

Cardiovascular monitoring

- Heart rate (HR), blood pressure (BP), respiratory rate (RR) and oxygen saturations will be continuously monitored throughout the procedure by the PACU nursing staff.

Recovery post ECT

- The patient is to remain in the PACU until fully recovered and for at least 30 minutes. The Anaesthetist will provide a handover to PACU nursing staff and notify of any special considerations.
- Nursing staff will:
 - * Regularly review and monitor patient's condition with special attention to airway management.
 - * Note and document patient's sedation/consciousness levels.
 - * Check the patient's mouth/teeth for any injury.
 - * Check ECT electrode sites for redness/blistering and signs of allergy.

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- * Manage the patient's recovery as per the PACU procedures.
- * Reassure patient and re-orientate to time and place.
- * Continue to monitor the patient for post ECT side effects (post anaesthetic side effects, potential post ictal delirium) taking appropriate action as required.
- * When the patient is fully recovered remove IV cannula.
- * Ensure all care is documented.
- * Handover the patient to the escort nurse, ensuring positive identification, medical history and any specific post ECT instructions.
- * The PACU nurse hands over the patient to ward staff utilising the Post-operative record (RGMR94.B). This form should be signed by the nurse giving handover and the nurse receiving handover
- * Notify the treating Consultant Psychiatrist/ Psychiatric Registrar / MO if any problems are noted and complete a Clinical Incident report on the DATIX system, as per the [DoH Clinical Incident Management Policy 2019](#). Serious incidents related to the practice of ECT must be reported promptly to the Chief Psychiatrist by the treating Consultant Psychiatrist before the end of the shift, as per the [Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist 2018](#).

Nursing management on return to the Mental Health Inpatient Unit post ECT

- Mental Health Unit nurse receives a handover from the PACU staff regarding the patient's treatment and condition.
- The Mental Health Unit nurse will escort the patient back from PACU utilising a wheelchair. Electric wheelchair will be utilised to meet bariatric patient needs.
- Patient will be offered food, fluids and medications as appropriate.
- Assessment of nausea, headache and muscle pain is to be carried out and appropriate medication administered as ordered by the MO.
- Advise the treating team Psychiatric Registrar/MO or intern/resident delegate to conduct a physical examination within 2-4 hours post ECT treatment. The MO will need to examine the patient and document the process in the patient's medical record.
- The patient's physical observations HR, BP, RR, oxygenation saturation, temperature, and neurological status are to be recorded immediately upon return to the ward every:
 - * 15 minutes for the first hour on the Adult Deterioration Detection System (ADDS) chart (RGMR141A).
 - * 30 minutes for the second hour post ECT, then observations may be ceased when the patient meets the following criteria:
 - Patient's conscious state must be awake, alert and orientated.
 - Patient's post ECT observations of HR, RR, BP are all stable and within normal limits, as established pre-ECT.
 - * Allocated nurse to ensure Post Electroconvulsive orientation assessment (RGMR94.H) is completed 30 minutes post ECT treatment if not already completed by the PACU nurse.

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- Re-orientate the patient to the ward environment. If orientation is significantly affected then the Treating Psychiatric Registrar/MO and ECT Coordinator must be notified, as the patient may be experiencing ongoing low-grade ictal activity.
- Patient is to remain on the ward for a minimum of 4 hours prior to leaving the ward after having ECT and the Treating Psychiatric Registrar/MO has to review the patient before leave is granted and document in the patients records.
- Ensure the patient is accompanied by support person when leaving the ward for and must remain in their care 24 hours post treatment.
- Instruct the patient not to drive or sign any legal documents for 24 hours post treatment.
- The ECT Coordinator will ensure that all ECT data is entered into the relevant database by the end of each shift.

ECT review post procedure

- The patient's Consultant Psychiatrist will review the patient following each treatment.
- The Treating Psychiatric Registrar/MO must conduct a safety check within 2-4 hours post ECT treatment on the ward. (Please use the Purple ECT Sticker in margin of medical record).
- The Consultant Psychiatrist is responsible for prescribing the on-going treatment.
- Documented review of the patient after each ECT treatment and recommendations on stimulus intensity and electrode placement for the next treatment by the treating team is recorded using the blue stamp ECT (located in Older and Adult Wards in the middle handover offices).
- The blue stamp can be completed by the Psychiatric Registrar/MO but the decision on its content is the Consultant Psychiatrist's responsibility.

ECT weekly patient reviews (black stamp)

- The black stamp 'ECT weekly review' is used to easily identify the comprehensive weekly review of the patient and evaluation at the end of the ECT course.
- The ECT review meeting is conducted on a weekly basis to
 - * Identify new patients requiring ECT.
 - * Review the treatment progress of existing patients receiving ECT.
- The Psychiatric Registrar/MO of the Treating team must attend this meeting and provide an update on the patient and document the discussion at the meeting in the medical record.
- The meeting attendees are: Consultant Psychiatrists, Treating team Registrars/MOs of those patients receiving ECT, NUM, or Clinical Nurse Specialist (CNS), ECT Coordinator, and when possible, the Pharmacist for medication reviews.

End of ECT course review and evaluation

- The Consultant Psychiatrist must ensure a full review and evaluation of the current course of ECT treatment is documented in the medical record.
- Documentation of a post ECT review following the course of treatment, encompassing response to ECT, adverse effects, management and recommendations for ECT treatment in the future.
- Documentation of the evaluation of the ECT course also to be included in the

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discharge summary to the General Practitioner (GP).

- On completion of ECT treatment for external patients referred to the service for ECT treatment, feedback will be provided to the external referrer by provision of a detailed discharge summary. Feedback will also include weekly documented PSOLIS reviews at weekly ward rounds.

ECT Nurse Coordinator

Responsibilities are to:

- Manage the ECT Suite by ensuring it is properly prepared, organised and maintained.
- Supervise the operation of ECT service sessions.
- Educate and advise patients, carers and other professionals about ECT.
- Ensure a program of education is included in the education calendar for the ward for mental health and surgical nursing staff involved in ECT treatment as well as inclusion of ECT as a topic at the Mental Health Journal Club twice a year.
- Management of ECT bookings and cancellations.
- Ensure the Thymatron® IV is maintained accurately and in good working order.
- Set up before commencement of any ECT procedures.
- Check all resuscitation equipment e.g. laryngoscopes all suction and oxygen are available and working.
- Discuss the order of treatment with the team involved in ECT treatment.
- Ordering and restocking ECT supplies in consultation with NUM.
- Develop/update operational policies and procedures related to ECT.
- Continuous quality improvement activities.
- Ensure effective management of complaints.
- Report serious incidents in relation to the practice of ECT to the OCP as per the Notifiable Events requirements under MHA 2014.
- Annual report of the ECT Service to the Office of Chief Psychiatrist.
- Ensure that all records are maintained and completed.
- Ensure that nursing staff providing ECT are trained and competent in the administration of ECT.
- Complete monthly report for the Office of Chief Psychiatrist (Form 13) and send to Co-Directors Mental Health, Program manager Mental Health In patient Unit, NUM.
- Provide a 6 monthly report to Mental Health Clinical Governance Sub-committee.

Consultant Psychiatrist in charge of ECT Services

- Ensure there is a functioning credentialing system for ECT.
- Ensure training and competency requirements are consistent with the Chief Psychiatrist Guidelines for the use of ECT in Western Australia.
- Ensure, along with nursing, that serious incidents are reported promptly to the Chief Psychiatrist.

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Clinical Incident Management

- Notify the treating Consultant Psychiatrist/Registrar if any problems are noted and complete a Clinical Incident report on the DATIX system, as per the [DoH Clinical Incident Management Policy 2019](#)
- It is a statutory requirement under the Mental Health Act 2014 ([MHA 2014](#)) that all notifiable incidents pertaining to psychiatric patients are reported to the Chief Psychiatrist as soon as practicable, ideally within 48 hours of the event.

Compliance monitoring

Compliance with this policy and procedure will be monitored via:

- Monthly statistics report to Office Chief Psychiatrist via MHA Form 13 and for emergency ECT as required
- Six monthly statistical report to Mental Health Clinical Governance sub committee
- Annual report of the ECT Service to Office of Chief Psychiatrist
- Annual audit will be carried out by the ECT Coordinator on at least the following:
 - * Documented consent, involuntary, emergency and voluntary.
 - * Pre-ECT checklists.
 - * ECT treatment forms.
 - * Side effects and complications.
 - * Basic treatment outcomes.
 - * Patient and family education activities.
 - * Patient satisfaction.

Stamps and sticker use

- For the Weekly ECT meeting the Black stamp is used for identification of when the meeting occurred.
- The Blue stamp is used after each ECT treatment to record stimulus intensity and electrode placement for the next treatment authorised by Consultant Psychiatrist.
- The Purple ECT sticker is to be used in the margin of the medical record when the entry is relevant to ECT. This may include a Post ECT review, a review for ECT side effects, or an ECT evaluation at end of a course of ECT.

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2. Mankad M, Beyer J, Weiner R, Krystal A. Clinical Manual of Electroconvulsive Therapy. Arlington, VA: American Psychiatric Publishing Inc; 2010. Available from: https://psychoreanimatology.org/download/books/Clinical_Manual_of_Electroconvulsive_Therapy_2010.pdf.

Links

[Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy 2015](#)

[Criminal Law \(Mentally Impaired Accused\) Act 1996](#)

[DoH Clinical Incident Management Policy](#)

[Mental Health Act 2014](#)

[Policy for Mandatory Reporting of Notifiable Incidents to the Chief Psychiatrist 2018](#)

[RGH Clinical Pathway for Novel Coronavirus COVID-19](#)

[RKPG Fasting for Pre-Operative Patients \(Acute\) Policy](#)

[RKPG Patient Identification](#)

[RKPG Patients' Property – Management in Mimidi \(Mental Health\) Guideline](#)

[RKPG Resuscitation Trolley \(Adult\) – Equipment and Checking \(Minimum Standard\) \(Acute\) Policy](#)

[Royal Australian and New Zealand College of Psychiatrists \(RANZCP\) professional practice guidelines for the administration of electroconvulsive therapy 2019](#)

[SMHS Credentialing and defining the scope of clinical practice for medical, nursing and midwifery practitioners](#)

[The E.C.T. Guide: The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2006](#)

[UpToDate: Technique for performing electroconvulsive therapy \(ECT\) in adults](#)

[Journal of ECT: Validation of the 10 item Orientation Questionnaire A new tool for monitoring post electroconvulsive therapy disorientation](#)

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National Standards



National Mental Health Standards



Chief Psychiatrist Standard for the administration of ECT ☒

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